

SM PHARMACEUTICALS SDN. BHD.

LORATEN (LORATADINE) TABLET 10 MG AND SYRUP 5 MG / 5 ML

DESCRIPTION:

Tablet: A white to off white, oblong shaped tablet with break line on one side.

Syrup: A clear syrup having peach flavour.

COMPOSITION:

Each tablet contains: Loratadine10 mg.

Each 5 ml syrup contains: Loratadine5 mg

ACTIONS AND MODE OR MECHANISMS OF ACTION:

Actions:

Loratadine is a non-sedative H₁-histamine receptor antagonist with anti-allergic properties, devoid of anti-cholinergic activity. It is rapidly effective and long-lasting, allowing once-a-day administration.

Mechanism of action:

Antihistamines used in the treatment of allergy act by competing with histamine for H₁-receptor sites on effector cells. They thereby prevent, but do not reverse, responses mediated by histamine alone. Antihistamines antagonize, in varying degrees, most of the pharmacological effects of histamine, including urticaria and pruritus. Also, the anti-cholinergic actions of most antihistamines provide a drying effect on the nasal mucosa.

PHARMACOLOGY (SUMMARY OF PHARMACODYNAMICS AND PHARMACOKINETICS):

Absorption

After oral administration, loratadine is rapidly and almost completely absorbed. Ingestion of food may enhance the absorption of loratadine by 40% and of its active metabolite by 15%.

Metabolism

Loratadine is extensively and rapidly metabolised in the liver to an active metabolite descarboethoxyloratadine.

Half -life

Reported half-lives for loratadine and descarboethoxyloratadine are 12 and 18 hours respectively.

Protein-binding

Loratadine is about 98% bound to plasma proteins; descarboethoxyloratadine is less extensively bound (73 - 76 %).

Time to peak concentration

Loratadine – 1 to 2 hours

Descarboethoxyloratadine (active metabolite) – 3 to 4 hours.

Time to peak effect

4 to 6 hours

Duration of action

At least 24 hours.

Elimination

It is excreted in urine (40% in 10 days) and faeces (42% in 10 days). 27% of the total dose is excreted in the urine in the conjugated form within 24 hours.

INDICATIONS:

Indicated for the relief of symptoms associated with allergic rhinitis, such as sneezing, nasal discharge (rhinorrhea) and itching, as well as ocular itching and burning.

It is also indicated for relief of symptoms and signs of chronic urticaria and other allergic dermatologic disorders.

CONTRAINDICATIONS:

Loratadine is contraindicated in patients who have shown hypersensitivity or idiosyncrasy to its component.

SIDE EFFECTS / ADVERSE REACTIONS:

Loratadine showed no clinically significant sedative or anti-cholinergic properties since fatigue, sedation, headache and dry mouth were rarely reported events. Alopecia has been reported rarely.

PRECAUTIONS / WARNINGS:

High risk groups

Pregnant woman

Although loratadine was not found to be teratogenic in animals, the safe use of loratadine during pregnancy has not been established and therefore the compound should be used only if the potential benefit justifies the potential risk to foetus.

Breast milk

Since loratadine is excreted in breast milk and because of the increased risk of antihistamines for infants, particularly newborns and premature infants, a decision should be made whether to discontinue nursing or discontinue the drug.

Children

Safety and efficacy of tablets in children < 6 years have not yet been established. Although adequate and well-controlled studies have not been done in the paediatric population, loratadine is not likely to cause anti-cholinergic or significant CNS effects in children.

Warning:

Drugs known to inhibit hepatic metabolism should be coadministered with caution until definitive interaction studies can be completed. The number of subjects who concomitantly received macrolide antibiotics, ketoconazole, cimetidine, ranitidine, or theophylline along with loratadine in controlled clinical trials is too small to rule out possible drug interactions.

DRUG INTERACTIONS:

In contrast to many other H₁-receptor antagonists, loratadine has no potentiating effects when administered concomitantly with alcohol, as measured by psychomotor performance.

Loraten should be discontinued approximately 48 hours prior to skin testing procedures since antihistamines may prevent or diminish otherwise positive reactions to dermal reactivity indicators.

Increase in plasma concentrations of loratadine has been reported after concomitant use with ketoconazole, erythromycin or cimetidine in controlled clinical trials but without clinically significant changes (including electrocardiographic).

Other drugs known to inhibit hepatic metabolism should be co-administered with caution until definitive interaction studies can be completed.

RECOMMENDED DOSAGE, DOSAGE SCHEDULE AND ROUTE OF ADMINISTRATION:

Route of administration: Oral.

Adults and children 12 years and over: 10 mg (1 tablet) once daily.

Patients with severe liver impairment should be administered with a lower initial dose because they may have reduced clearance of loratadine; an initial dose of 5 mg or 5 mL once daily or 10 mg or 10 mL every other day is recommended.

Children 2 – 12 years:

Body weight > 30 kg: 10 mg (10 ml syrup) once daily.

Body weight ≤ 30 kg: 5 mg (5 ml syrup) once daily).

SYMPTOMS AND TREATMENT FOR OVERDOSE, AND ANTIDOTE(S):

Symptoms:

Anti-cholinergic and CNS stimulant effects are more likely to occur in children with overdose. Hypotension may also occur in the elderly at usual doses.

Anti-cholinergic and CNS effects may be less likely to occur with loratadine than with the first-generation anti-histamines.

Treatment:

Since there is no specific antidote for overdose with antihistamines, treatment is symptomatic and supportive.

To decrease absorption-

Induction of emesis (syrup of ipecac recommended); however, precaution against aspiration is necessary, especially in infants and children. Following emesis, adsorption of any drugs remaining in the stomach may be attempted by the administration of activated charcoal as a slurry with water.

Gastric lavage (isotonic or 0.45% sodium chloride solution) if patient is unable to vomit within 3 hours of ingestion.

To enhance elimination-

Saline cathartics (milk of magnesia) are sometimes used.

Specific treatment-

Vasopressors to treat hypotension; however, epinephrine should not be used since it may further lower blood pressure.

Oxygen and intravenous fluids.

Precaution against use of stimulants (analeptic agents) because they may cause seizures.

DOSAGE FORM(S) AND STRENGTH(S) AVAILABLE:

Tablet: 10 mg

Syrup: 5 mg per 5 ml

PACKING / PACK SIZES:

Tablet: Blister pack of 3 x 10's and 10 x 10's

Syrup: Bottles of 60 ml and 100 ml.

STORAGE CONDITIONS, USER INSTRUCTIONS AND PHARMACEUTICAL PRECAUTIONS:

STORE IN A DRY PLACE, BELOW 30°C. PROTECT FROM LIGHT AND FREEZING.

SHELF LIFE: 3 years

MANUFACTURER AND PRODUCT REGISTRATION HOLDER:

SM PHARMACEUTICALS SDN BHD (218620-M)

Lot 88, Sungai Petani Industrial Estate

08000 Sungai Petani,

Kedah Darul Aman,

Malaysia.

06.01.2023